

1. Administrative & Study Identification

Full Study Title: A Phase IIIB Study to Evaluate the Use of Capivasertib in Combination With Fulvestrant in Patients With Advanced Breast Cancer Who Have Relapsed/Progressed on ET and CDK4/6 Inhibitor Reflecting Real World Clinical Practice in Spain

Official Title: A Phase IIIB Study to Evaluate the Use of Capivasertib in Combination With Fulvestrant in Patients With HR+ / HER2- Advanced Breast Cancer Who Have Relapsed/Progressed on ET and CDK4/6 Inhibitor Reflecting Real World Clinical Practice in Spain

Short Title/Acronym: CAPItana **Protocol Number:** D3612L00005 **NCT Number:** NCT06764186 **Posting ID:** 590845935107963993

Trial Status: RECRUITING **Sponsor Name:**

AstraZeneca **Investigational Product:** Capivasertib + Fulvestrant

Phase of Development: Phase III (PHASE3) **Medical Monitor:**

AstraZeneca Clinical Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effectiveness and safety of capivasertib + fulvestrant treatment administration, primarily measured by Time to Next Treatment (TTNT), in patients with locally advanced (inoperable) or metastatic HR-positive (HR+) / HER2-negative breast cancer with PIK3CA/AKT1/PTEN-altered tumors following recurrence or progression on or after endocrine therapy (ET) and CDK4/6 inhibitor. **Secondary Objectives:** To evaluate the number of patients with adverse events (AEs), Time to first Subsequent Chemotherapy (TFSC), and Progression-Free Survival (PFS).

2.2 Background & Rationale To address the clinical need for effective post-relapse management in patients with HR+/HER2-early breast cancer who have progressed during or after adjuvant therapy with CDK4/6 inhibitors (CDK4/6i) and endocrine therapy (ET). Capivasertib, a potent pan-AKT kinase inhibitor, in combination with fulvestrant, has demonstrated a significant reduction in the risk of disease progression or death compared to fulvestrant alone in patients with HR+/HER2- advanced breast cancer. This study aims to generate real-world evidence for this combination regimen specifically in the Spanish clinical practice setting for patients harboring PIK3CA/AKT1/PTEN alterations.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Single Group Assignment) **Blinding:** Open-label (Masking: None) **Randomization:** N/A

3.2 Planned Enrollment & Duration Trial Status: RECRUITING **Target N:** 100 participants **Number of Sites:** Multicenter study in Spain **Estimated Study Start:** 07-Jan-2025 **Estimated Primary Completion:** 31-Dec-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically confirmed HR+/HER2- breast cancer (primary or metastatic): HR+ defined as ER+ with or without PR+; HER2- defined as IHC 0 or 1+, or IHC 2+/ISH-. 2. Patient with tumours harbouring at least one PIK3CA/AKT1/PTEN qualifying alteration detected by a validated test (including NGS on tissue, cell block, or if tissue/cell block is not available, on ctDNA, as per protocol requirements. If alteration is initially detected by a method other than NGS, NGS on tissue/cell block must be performed within 45 days unless not available). 3. Metastatic or locally advanced disease with radiological or objective evidence of recurrence or progression. Patients must have received treatment with an ET in combination with CDK4/6i and have: Radiological evidence of breast cancer recurrence or progression while on, or within 12 months of the end of (neo)adjuvant treatment with an ET with CDK4/6i, OR Radiological evidence of progression while on prior ET with CDK4/6i administered as a treatment line for locally advanced or metastatic breast cancer. 4. Eastern Cooperative Oncology Group (ECOG)/ World Health Organisation (WHO) performance status ≤ 2 at enrollment (not more than 20% of patients with ECOG PS2 will be allowed). 5. Reproduction: Women of childbearing potential (WOCBP) patients with ovarian suppression induced by LHRH agonist should agree to use 2 forms of highly effective methods of accepted contraception to prevent pregnancy. Male patients should use barrier contraception. 6. Informed consent provided.

4.2 Exclusion Criteria 1. Unknown or non-altered PIK3CA/AKT1/PTEN-status. 2. Prior/concomitant therapy restrictions: More than 2 lines of endocrine therapy or in combination with CDK4/6i for inoperable locally advanced or metastatic disease; More than 1 line of chemotherapy for inoperable locally advanced or metastatic disease. Adjuvant and neoadjuvant chemotherapy are not classed as lines of chemotherapy for ABC. 3. Severe or uncontrolled systemic diseases, uncontrolled hypertension, active infections including hepatitis B, hepatitis C, HIV, and confirmed COVID-19. 4. History of another primary malignancy except for malignancy treated with curative intent with no known active disease ≥ 2 years before the first dose of study intervention and of low potential risk for recurrence. 5. Disease burden

making the patient ineligible for endocrine therapy per the investigator judgement. 6. Unresolved toxicities from prior therapy greater than CTCAE grade 1. 7. Leptomeningeal metastases or symptomatic, unstable, or steroid-dependent brain metastases. 8. HbA1c $\geq$ 8.0% (63.9 mmol/mol). 9. Inadequate bone marrow reserve or organ function. 10. Known abnormalities in coagulation. 11. Refractory nausea, vomiting, malabsorption syndrome, chronic gastrointestinal diseases, inability to swallow formulated product, or significant bowel resection. 12. Previous allogeneic bone marrow or solid organ transplant. 13. Known immunodeficiency syndrome. 14. Evidence of dementia altered mental status or any psychiatric condition. 15. Pregnant women. 16. Participants with significant QT interval prolongation or a history of related cardiac conditions, including arrhythmias or recent cardiac procedures. 17. Participation in another clinical study with a study intervention.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Capivasertib will be administered as 400mg BD (twice daily), 4 days on 3 days off. This is in combination with fulvestrant at the approved dose of monthly 500mg (2 × 5mL IV), with an additional loading dose in Cycle 1. **Route of Administration:** Oral (Capivasertib) and Intravenous (IV) Injection (Fulvestrant). **Duration of Treatment:** Until objective disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications for adverse event management. **Prohibited:** Concurrent treatment with AKT1, PIK3CA, and mTOR inhibitors is not allowed. Strong CYP3A inhibitors/inducers may require an adequate washout period or dose reduction.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Mutational Status (NGS), Baseline Imaging (RECIST 1.1)
Baseline	Day 0	First Dose (Capivasertib + Fulvestrant), Baseline Safety Labs
Interim	Q4W/Q8W	Efficacy Assessment (Tumor Imaging), Safety Labs, AE Monitoring
End of Study	Variable	End of Treatment Visit, Final Vital Signs, Exit Interview, Survival Follow-up

(Note: Real-world visit schedules reflect standard Spanish clinical practice intervals as directed by treating oncologists).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Time to next treatment (TTNT). Time to next treatment (TTNT1) is defined as the time from the date of first dose of capivasertib + fulvestrant until the first subsequent anti-cancer therapy after discontinuation of study treatment or death due to any cause. **Measurement Method:** Documented via electronic Case Report Forms based on real-world treatment records.

7.2 Secondary & Exploratory Endpoints 1. Number of patients with adverse events (AEs). 2. Time to first Subsequent Chemotherapy (TFSC). 3. Progression-free survival (PFS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection from signed informed consent through the defined post-treatment follow-up period. Graded using CTCAE criteria. **Serious Adverse Events (SAEs):** Required to be reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Utilizes standard Sponsor-established safety monitoring protocols.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy endpoints. Safety Set for all patients receiving at least one dose of the investigational product. **Sample Size Justification:** $N=100$ powered to detect clinically meaningful efficacy (TTNT) in the Spanish real-world subgroup. **Handling of Missing Data:** Standard survival analysis methods including censoring at last known alive/progression-free date for time-to-event endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Centralized and onsite data monitoring reflecting the real-world operational strategy in Spain. **Audit Trail:** Electronic Case Report Form (eCRF) utilizing 21 CFR Part 11 compliant data capture and query systems.

11. Study Identifiers & Governance

Primary ID: D3612L00005 **Registry Number:** NCT06764186 **Posting ID:** 590845935107963993 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** CAPItana **Official Regulatory Title:** A Phase IIIB Study to

Evaluate the Use of Capivasertib in Combination With Fulvestrant in Patients With HR+ / HER2- Advanced Breast Cancer Who Have Relapsed/Progressed on ET and CDK4/6 Inhibitor Reflecting Real World Clinical Practice in Spain **Trial Status:** RECRUITING

12. Clinical Framework (Breast Cancer Specific)

Primary Condition: Locally Advanced or Metastatic Breast Cancer
Diagnosis Threshold: HR+ / HER2- with PIK3CA/AKT1/PTEN qualifying alterations
Medical Methodology: Targeted combination endocrine therapy
Optimization Target: Disease control and extension of time to next treatment

13. Study Procedures & Methodology

Management Pathway: Targeted oncology management for relapsed HR+/HER2- ABC
Education Protocol (HCP): Investigator protocol training at site initiation
Education Protocol (Patient): Standard patient information leaflets and oral medication diaries
Quality Audit Mechanism: Periodic remote and onsite data verification of clinical measurements and mutational assays

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until data cutoff (Estimated late 2027)
Onsite Visit Cadence: Aligned with standard oncologic care cycles (e.g., Q4W/Q8W)
Remote Monitoring Frequency: Continuous survival follow-up post-treatment
Checklist Review Interval: Routine clinical assessment intervals

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				